

Important information. Please read carefully.

# Bencovee

## Cream

### COMPOSITION

Contains 1% w/w Clotrimazole and 0.05% w/w Betamethasone (as Betamethasone Dipropionate).

Methyl Hydroxybenzoate 0.18% w/w and Propyl Hydroxybenzoate 0.02% w/w as preservatives.

### PHARMACODYNAMICS

Bencovee cream contains clotrimazole which is an imidazole antifungal agent and betamethasone dipropionate, a glucocorticoid exhibiting the general properties of corticosteroids.

Clotrimazole is a broad-spectrum antifungal agent with activity against Trichomonas, Staphylococci and Bacteroides.

Topical corticosteroids are effective in the treatment of a range of dermatoses because of their anti-inflammatory antipruritic and vasoconstrictive actions.

### PHARMACOKINETICS

When applied topically clotrimazole penetrates the epidermis but there is little if any systemic absorption. Clotrimazole is metabolized in the liver to inactive compounds and excreted in the feces and urine.

Betamethasone is readily absorbed when applied topically. Sufficient amount of betamethasone may be absorbed to give systemic effect when applied under occlusive dressing or on broken skin. Betamethasone is mainly metabolized in liver and excreted in the urine.

### INDICATIONS

Bencovee cream is indicated for the topical treatment of dermal infections such as tinea pedis, tinea cruris and tinea corporis due to *Trichophyton rubrum*, *Trichophyton mentagrophytes*, *Epidermophyton floccosum* and *Microporum canis*; and also candidiasis due to *Candida albicans*.

## **DOSAGE AND ADMINISTRATION**

For topical use only.

Gently massage sufficient Bencovee Cream into the affected and surrounding skin areas twice a day, in the morning and evening, for two weeks in tinea cruris, tinea corporis and candidiasis, and for four weeks in tinea pedis.

### *Duration Of Therapy:*

Clinical improvement, with relief of erythema and pruritus, usually occurs within the first three to five days of treatment.

If a patient with tinea cruris, tinea corporis, or candidiasis shows no clinical improvement after one week of treatment with Bencovee Cream, the diagnosis should be reviewed.

In tinea pedis, the treatment should be applied for two weeks prior to making that decision.

If the condition persists after two weeks in tinea cruris and tinea corporis, and after four weeks in tinea pedis, treatment with Bencovee Cream should be discontinued. Alternate therapy may then be instituted with an appropriate antifungal agent only.

The use of Bencovee Cream for longer than four weeks is not recommended.

## **CONTRAINDICATIONS**

Known hypersensitivity to Clotrimazole and Betamethasone Dipropionate or other imidazoles and corticosteroids.

Treatment should be discontinued and appropriate therapy instituted if irritation or sensitization develops.

Contraindicated in facial rosacea, acne vulgaris, perioral dermatitis, napkin eruptions and bacterial or viral infections.

## **WARNINGS AND PRECAUTIONS**

Not for ophthalmic use.

Should not be used with occlusive dressing.

If irritation or sensitization develops with the use of this product, treatment should be discontinued.

In the presence of a bacterial infection, an appropriate antibacterial agent should be administered concomitantly. If a favourable response does not occur promptly, this product should be discontinued until the infection has been controlled adequately.

Systemic absorption of topical corticosteroids will be increased with the use of more potent corticosteroid agents, with prolonged usage or if extensive body surface areas are treated. Therefore, patients receiving large doses of potent topical corticosteroids applied to a large surface area should be evaluated periodically for evidence of hypothalamic pituitary adrenal (HPA) axis suppression. If HPA axis suppression occurs, an attempt should be

made to withdraw the drug to reduce the frequency of application, or to substitute with a less potent corticosteroid agent.

Visual disturbance may be reported with systemic and topical corticosteroid use. If a patient presents with symptoms such as blurred vision or other visual disturbances, the patient should be considered for referral to an ophthalmologist for evaluation of possible causes which may include cataract, glaucoma or rare diseases such as central serous chorioretinopathy (CSCR) which have been reported after use of systemic and topical corticosteroids.

Bencovee cream contains:

Cetostearyl alcohol which may cause localised skin reactions (e.g. contact dermatitis).

Propylene glycol which may cause skin irritation. Because this medicine contains propylene glycol, do not use it on open wounds or large area of broken or damaged skin (such as burns).

#### Pediatric:

This product is not recommended to be used in diaper dermatitis.

Long term continuous therapy should be avoided in all children irrespective of age.

If used on children, courses should be limited to 5 days.

### **DRUG INTERACTIONS**

There are no known interactions.

### **PREGNANCY AND LACTATION**

#### Pregnancy

Since safety of topical corticosteroid use in pregnant women has not been established, drugs of this class should be used during pregnancy only if the potential benefit justifies the potential risk of the foetus. Drugs of this class should not be used extensively in large amounts or for prolonged periods of the time in pregnant patients.

#### Lactation

Since it is not known whether topical administration of corticosteroids can result in sufficient systemic absorption to produce detectable quantities in breast milk, a decision should be made to discontinue nursing or to discontinue the drug, taking into account the importance of the drug to the mother.

### **SIDE EFFECTS**

Adverse reactions reported for combination product of clotrimazole and betamethasone dipropionate include: burning and stinging, maculopapular rash, oedema, paraesthesia and secondary infection.

Reported reactions to clotrimazole include erythema, stinging, blistering, peeling, oedema, pruritus, urticaria and general irritation of the skin.

Reactions to betamethasone dipropionate include: burning, itching, irritation, dryness, folliculitis, hypertrichosis, acneiform eruptions, hyperpigmentation, hypopigmentation, perioral dermatitis, allergic contact dermatitis, maceration of the skin, secondary infection, skin atrophy, striae miliaria, capillary fragility (ecchymoses), blurred vision and sensitisation. Blurred vision can also occur.

In children receiving topical corticosteroids, Hypothalamic-pituitary adrenal (HPA) axis suppression, Cushing's syndrome and intracranial hypertension have been reported. (See Warnings and Precautions).

### **SYMPTOMS AND TREATMENT FOR OVERDOSAGE**

Excessive or prolonged use of topical corticosteroids can suppress pituitary-adrenal function, resulting in secondary adrenal insufficiency, and produce manifestations of hypercorticism including Cushing's disease.

Appropriate symptomatic treatment is indicated. Acute hypercorticoid symptoms are usually reversible. Treat electrolyte imbalance, if necessary. In case of chronic toxicity, slow withdrawal of corticosteroids is advised.

### **STORAGE CONDITION**

Store below 25°C.

### **SHELF LIFE**

The expiry date is indicated on the packaging.

### **PRODUCT DESCRIPTION, DOSAGE FORM AND PACKAGING AVAILABLE**

White, smooth, homogeneous cream.

Available in pack size of 15g.

For further information, please consult your physician or pharmacist.

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Manufacturer and Product Registration Holder (in Malaysia)

**Xepa-Soul Pattinson (Malaysia) Sdn Bhd**

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